

Lumbar Spine Surgery

ORG: S-2810 (HC)

[Link to Codes](#)

MCG Health

Home Care

28th Edition

This MCG guideline has not yet been updated to reflect new guidance and requirements found in the 2024 Medicare Advantage and Part D Final Rule (CMS-4201-F). For Medicare Advantage beneficiaries, before referring to MCG's guidelines users should first follow all guidance related to this topic located in CMS National Coverage Determinations (NCDs), Local Coverage Determinations (LCDs), and other statutes and regulations. Content used in MCG guidelines may be used to supplement but does not replace, modify, or supersede existing Medicare regulations or applicable NCDs or LCDs.

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Clinical Indications for Admission to Home Healthcare

- Home healthcare is/was needed for appropriate care of the patient because of **ALL** of the following[A][B](1)(2)(3):
 - The patient has had a face-to-face encounter with a physician or allowed non-physician practitioner (NPP) no more than 90 days prior to (or within 30 days after) the start of care related to the home health service needs.[C](4)
 - The patient is Homebound or homebound waiver applies (eg, accountable care organization).(5)(6)
 - The patient's needs can be safely met with Intermittent home care.
 - The patient needs education on condition and self-management care with teach-back.(7)
 - Skilled care at home is required for **1 or more** of the following(8):
 - Cast or immobilizer care(9)(10)
 - Gastrointestinal status assessment(11)
 - Genitourinary status assessment(11)
 - Home safety assessment(12)(13)
 - Infusion (IV) therapy management (eg, insertion site use and care, infusion administration, monitoring reaction)(14)
 - Medication management, adherence instruction, and side effects assessment(15)
 - Neurologic status assessment(11)
 - Neurovascular status assessment(11)
 - Nutrition and hydration management(16)
 - Pain management(17)(18)
 - Psychosocial assessment, management, and referrals(19)
 - Rehabilitation therapy or equipment coordination[D][E][F][G][H](24)
 - Significant chronic condition exacerbation with need for clinical intervention and monitoring(25)
 - Skin integrity assessment(10)
 - Wound or dressing management(18)

Visit Data

Home Care Use for Condition

Home care use for condition refers to the frequency with which home care services are provided after inpatient hospitalization for this diagnosis or procedure: Rare: less than 5%; Occasional: from 5% up to 20%; Frequent: from 20% up to 40%; Very Frequent: 40% or greater.

Description	Commercial	Medicare
Home care use for this condition	Occasional	Frequent

Registered Nurse Visit Goal Table

Registered nurse visits are displayed as percentiles that are based on the observed utilization of registered nurse visits for this diagnosis. Individual patients may require fewer or more visits as appropriate for their clinical status and care needs. This table is designed to allow organizations to define their goals for registered nurse utilization by choosing from the displayed range.

Commercial RN Visits					Medicare RN Visits				
10%ile	20%ile	30%ile	40%ile	50%ile	10%ile	20%ile	30%ile	40%ile	50%ile
1	2	2	3	4	2	2	3	3	4

Rehabilitation Therapy Visits Table

Rehabilitation therapy visits are displayed as percentiles that are based on the observed utilization of therapy visits for this diagnosis. Individual patients may require fewer or more visits as appropriate for their clinical status and care needs. This table is designed to allow organizations to define their goals for home healthcare rehabilitation therapy utilization by choosing from the displayed range.

Type of Visit	Commercial Visits					Medicare Visits				
	10%ile	20%ile	30%ile	40%ile	50%ile	10%ile	20%ile	30%ile	40%ile	50%ile
PT	1	2	3	4	5	2	3	4	5	6
OT	1	1	1	1	1	1	1	1	1	2
SLP	-	-	-	-	-	-	-	-	-	-

A dash ("-") in the table indicates that there are not enough data available or the type of visit is not appropriate for this diagnosis.

Evaluation and Treatment

General Treatment Course

Stage	Clinical Status	Interventions
1	Clinical indications for admission met - Home care comprehensive assessment and initial home evaluation complete^[1] QM(20)(26) - The comprehensive assessment should be completed within 5 days after the start-of-care date. - If skilled nursing is ordered, nursing should complete start of care and confirm eligibility. - A qualified rehabilitation therapist (ie, PT, OT, SLP) may open and case manage home care if the orders are for therapy only (if any nursing care is ordered, the case is not considered therapy only). However, Medicare benefits do not cover cases if only occupational therapy (OT) is ordered. - Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment  SR complete for evaluation of current level of functioning (eg,  Activities of Daily Living (ADL) Scoring Tool Calculator)	☐ Establish and implement interdisciplinary care plan; clinical goals may include QM (31)(32): - Activity tolerance/physical mobility improved - Bladder status improved or stable - Bowel status improved or stable - Cognitive status improved or stable - Home safety improved or stable

- Psychosocial Assessment  SR complete **QM**(27)
- Social determinants of health screening complete. See Social Determinants of Health Screening Tool  SR for more information. **QM**
- Medication reconciliation complete. See Medication Reconciliation Tool  SR for more information. **QM**
- Home safety assessment complete. See Home Safety Assessment  SR for more information.
- Hospital readmission risk factor evaluation **QM** [J][K][L](28)
- Physical therapy (PT) initial home evaluation as ordered
- Occupational therapy (OT) initial home evaluation as ordered
- Skilled healthcare needs identified

- Infection risk reduced[M] **QM**(33)
- Knowledge deficit improved or resolved
- Laboratory values improved or within normal limits
- Medication regimen established
- Nutrition and hydration status improved or stable
- Pain improved
- Patient care management improved or stable
- Psychosocial issues improved or resolved
- Skin integrity improved or stable
- Tissue perfusion improved or stable(34)
- Vital signs stable
- Wound healing without complications
- Initiate transition of care planning. **QM**

-  Provide treatments and monitoring **QM** (26)(35):
- Bladder management(36)
 - Bowel management(18)
 - Cast or immobilizer management(37)(38)
 - Chronic condition management(39)
 - Compression stocking management(18)(40)
 - Fall risk management **QM** (41)
 - Infusion (IV) therapy management (eg, fluids, antibiotics, parenteral nutrition)(42)
 - Injection medication management (ie, intramuscular, subcutaneous)
 - Laboratory testing or monitoring, if ordered
 - Medication management **QM**
 - Nutrition management(36)
 - Pain management[N] **QM**(44)(45)
 - Pressure injury risk management **QM** (46)
 - Psychosocial issue and risk management **QM**
 - Thromboprophylaxis management(47)
 - Wound management(48)
 - Report complications to treating provider, if indicated.
 - Coordinate follow-up with treating provider(s).

		• Provide patient and caregiver education, reinforcement, and teach-back. QM (49) ◦ Review patient management techniques (eg, medication, activity, monitoring). ◦ Review complications and when and to whom to report. ◦ See corresponding Patient Education for Clinicians for more information. [-] Initiate referrals as indicated.(50) ◦ Physical therapy (PT) referral ◦ Occupational therapy (OT) referral ◦ Social worker referral ◦ Home Health Aide (HHA) referral ◦ Financial, for follow-up care, medication, and transportation ◦ Self-management information(51) ◦ Tobacco use treatment, as needed QM (52) ◦ Vocational rehabilitation(53)(54) ◦ Other
2	• Therapeutic response to interventions QM • Patient/caregiver able to monitor and self-manage condition, or unwilling or unable to be trained after sufficient efforts made (55) • Rehabilitation program established and patient participating as able with evaluation of current level of functioning (eg,  Activities of Daily Living (ADL) Scoring Tool Calculator) QM • Physical therapy (PT) progression • Occupational therapy (OT) progression	• Interdisciplinary care plan continues • Ongoing assessment of clinical needs [-] Provide treatments and monitoring QM : ◦ Bladder management ◦ Bowel management ◦ Cast or immobilizer management ◦ Chronic condition management ◦ Compression stocking management ◦ Fall risk management QM ◦ Infusion (IV) therapy management (eg, fluids, antibiotics, parenteral nutrition) ◦ Injection medication management (ie, intramuscular, subcutaneous) ◦ Laboratory testing or monitoring, if ordered ◦ Medication management QM ◦ Nutrition management ◦ Pain management QM ◦ Pressure injury risk management QM

		◦ Psychosocial issue and risk management QM ◦ Thromboprophylaxis management ◦ Wound management • Continue patient and caregiver education, reinforcement, and teach-back. QM ◦ Assess patient management ability and techniques. ◦ Provide instructional support as needed. ◦ Verify understanding of complications and when and to whom to report. • Report complications to treating provider, if indicated. • Continue transition of care planning. QM • Verify follow-up with treating provider. • Follow up on referral status, as indicated.
3	• IV therapy discontinued or can be managed independently • Inserted or implanted device discontinued, or functioning normally and can be managed independently(14) • Medical equipment and supplies available and safe use demonstrated(12) • Medical status stable for patient's condition(26) • Medication regimen established and reconciliation completed QM • Patient/caregiver able to monitor and self-manage condition independently, or unwilling or unable to be trained after sufficient efforts made(55) • Patient Safe at home or appropriate referrals made QM(56) • Psychosocial issues adequately addressed or appropriate referrals made QM • Rehabilitation program completed for safe transfer to lower level of care or patient no longer demonstrating significant functional gains (eg,  Activities of Daily Living (ADL) Scoring Tool Calculator)(57) • Significant chronic condition exacerbation resolved • Transition plans understood • Wound(s) healing without complications, or complications resolving	• Transition of care planning completed QM • Discharge from home care services

Recovery Milestones are indicated in **bold**.

Extended Visits

Additional skilled visits may be: brief (1 visit), moderate (2 to 3 visits), or prolonged (more than 4 visits).

- Additional nursing visits may be needed for **1 or more** of the following:
 - ☐ Failure to meet discharge criteria (Recovery Milestones within final stage of General Treatment Course)
 - Expect brief visit extension
 - ☐ Initiate Home Safety Assessment  SR due to change in patient's ability to perform activities of daily living (ADL) or instrumental activities of daily living (IADL).(12)
 - Expect brief visit extension.
 - ☐ Infusion therapy administration and assessment; examples include(14)(58):
 - New intravenous medication until safe tolerance established
 - Complications of infusion therapy (eg, extravasation of vesicant)
 - Central venous access device (CVAD) sterile dressing change unmanageable by patient or in outpatient setting
 - Peripheral IV access initiation or site rotation
 - Administration of agent requiring on-site monitoring by clinical personnel
 - Transition from parenteral to enteral nutrition or oral feeding, or from enteral nutrition to oral feeding

- Infusion therapy unmanageable by patient or caregiver while intense and complex care needs make home care safer and more practical than outpatient infusions
- Expect brief to moderate visit extension.
- ☐ Ongoing education required for patient or caregiver as indicated by demonstrated ability, willingness to learn, and(7)(26):
 - Need for improved understanding of patient's medical condition, diagnosis, disease, or disability
 - Need for improved understanding of methods and means to manage aspects of medical condition
 - Need for interventions to increase adherence to therapy
 - Expect brief to moderate visit extension.
- ☐ Pain evaluation and management; examples include(32)(44)(59):
 - Frequent pain assessment using validated diagnostic tool[O]
 - Early referral and treatment of pain
 - Ongoing education and training of pain treatment plan (eg, multimodal pain management)[P][Q]
 - Pain medication side effects or risk factors for adverse effects (eg, constipation, sedation, falls)
 - Expect brief to moderate visit extension.
- ☐ Possible fecal disimpaction due to change in bowel habits (eg, fecal impaction, inability to defecate, neurogenic bowel dysfunction)(18)(60)
 - Expect brief visit extension.
- ☐ Significant chronic disease exacerbation, with need for skilled clinical intervention and monitoring(26)
 - Expect brief visit extension.
- ☐ Wound management; examples include(18)(61):
 - Wound larger, or change in appearance (eg, appearance of eschar or new purulent drainage) since last assessment
 - Wound status change, with new type of dressing or procedure
 - New wound development (eg, pressure injury)
 - Topical negative pressure dressing change, or pump malfunction, if used
 - Expect brief to moderate visit extension.
- ☐ Additional PT visits may be needed for **1 or more** of the following(62)(63)(64):
 - Unanticipated functional problems impacting safe completion of mobility-related ADL (MRADL)(65)
 - Evaluation and retraining on use of assistive technology or assistive device
 - Exacerbation or new comorbidities impairing functional improvement
 - Evaluation and retraining of precautions to ensure safe completion of MRADL
 - Impaired cognition that prolongs teaching
 - Evaluation and retraining of home exercise program due to environmental change
 - Uncontrolled pain impacting learning process and participation
 - Caregiver training needed with adequate understanding and return demonstration
- ☐ Additional OT visits may be needed for **1 or more** of the following(66):
 - Unanticipated functional problems impacting safe completion of ADL(65)
 - Evaluation and retraining on use of adaptive equipment, assistive technology, or assistive device
 - Exacerbation or new comorbidities impairing functional improvement
 - Evaluation and retraining on precautions to ensure safe completion of ADL
 - Impaired cognition that prolongs teaching
 - Evaluation and retraining on home management program due to environmental change
 - Uncontrolled pain impacting learning process and participation
 - Caregiver training needed with adequate understanding and return demonstration

Patient Education

- Patient and caregiver education. See:
 - Lumbar Discectomy, Foraminotomy, or Laminotomy: Patient Education for Clinicians [SR](#)
 - Lumbar Fusion: Patient Education for Clinicians [SR](#)
 - Lumbar Laminectomy: Patient Education for Clinicians [SR](#)

Discharge Planning

- Transition of care planning may include(26)(56):
 - ☐ Assessment and early establishment of anticipated discharge destination and plans for care, including:
 - Treatment plan development involving multiple providers
 - Premorbid functioning(67)
 - Patient and caregiver preferences and abilities
 - Psychosocial status. See Psychosocial Assessment [SR](#) for further information. **QM**
 - Social determinants of health screening. See Social Determinants of Health Screening Tool [SR](#) for further information. **QM(68)**

- Acceptance for care for patient at next level of care, as appropriate
- Transition of care plan complete, which may include:
 - Patient and caregiver education complete. See Teach Back Tool [SR](#) for further information.(7)
 - ☐ Medication reconciliation completion includes **QM** (69)(70):
 - Compare patient's discharge list of medications (prescribed and over-the-counter) against provider's admission or transfer orders.
 - Assess each medication for correlation to disease state or medical condition.
 - Report medication discrepancies to prescribing provider, attending physician, and primary care provider, and ensure accurate medication order is identified.
 - Provide reconciled medication list to all treating providers.
 - Confirm that patient or caregiver can acquire medication.
 - Educate patient and caregiver.
 - Provide complete medication list to patient and caregiver.
 - Importance of presenting personal medication list to all providers at each care transition, including all provider appointments
 - Reason, dosage, and timing of medication (eg, use "teach-back" techniques)(15)
 - Encourage communication between patient, caregiver, and pharmacy for obtaining prescriptions, setting up home medication delivery, and reviewing for drug-drug interactions.
 - See Medication Reconciliation Tool [SR](#) for more information.
 - ☐ Appointments planned or scheduled:
 - Primary care provider
 - Outpatient imaging, tests, or procedures
 - Outpatient rehabilitation therapy (eg, PT)
 - Specialists for management of comorbidities as needed
 - Surgeon(71)
 - Other
 - Assistance and support referrals completed
 - ☐ Medical equipment and supplies coordinated (ie, delivered or delivery confirmed), as indicated:
 - ADL-supportive equipment
 - Cardiac care equipment and supplies (eg, compression stockings)(72)
 - Medication equipment and supplies
 - Orthopedic equipment
 - Wound care equipment and supplies
 - Other
 - Transition plan communicated to all members of patient's care team **QM**

Quality Measures

- HEDIS Measures include(73):
 - Medication reconciliation is completed.
 - Transition of care needs are assessed and addressed.
 - Psychosocial status (eg, depression screening) is assessed and addressed.
 - Fall risk is assessed and addressed.
 - Tobacco use is assessed and addressed.
 - Immunization status is assessed and addressed.
 - Readmission risk is assessed and addressed.
 - Social needs screening and intervention is completed.
- Home Health Compare Quality Measures include(74):
 - ADL status is assessed for improvement.
 - Readmission risk is assessed and addressed.
 - Pressure injury risk is assessed and addressed.
 - Oral medication management is assessed for improvement.
 - Timely initiation of care is assessed and addressed.
 - Medication reconciliation and education are completed.
 - Immunization status is assessed and addressed.
 - Dyspnea is improved.
 - Discharge to community is completed.
- Home Health Quality Reporting Program quality measures include(75):
 - ADL status is assessed for improvement.
 - Immunization status is assessed and addressed.
 - Medication reconciliation and education are completed.
 - Readmission risk is assessed and addressed.

- o Fall risk is routinely assessed.
 - o Pain status is assessed and pain is addressed.
 - o Oral medication management is assessed for improvement.
 - o Pressure injury risk is assessed and addressed.
 - o Timely initiation of care is assessed and addressed.
 - o Transfer of health information to provider and patient is completed.
 - o Dyspnea is improved.
- The Joint Commission Home Care National Patient Safety Goals include(76):
 - o Medication reconciliation is completed. Education is provided on medication administration including the importance of bringing up-to-date medication lists to all provider visits.
 - o Healthcare-associated infection risk is reduced.
 - o Fall risk is assessed and addressed.

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Footnotes

[A] **Evidence:** A retrospective cohort study based on 1151 Medicare accountable care organization (ACO) patients suggests utilizing a transitional care planning approach to prioritize discharging patients to the least restrictive next level of care, as patients discharged to skilled nursing facilities were almost 5 times more likely to be readmitted to the hospital at 30 days compared to patients discharged to the home healthcare setting.(1) [A in Context Link 1]

[B] **Evidence:** A retrospective cohort study of 25,357 older patients discharged from a skilled nursing facility (SNF) found that patients who received services from a Medicare-certified home health agency had decreased odds of rehospitalization or SNF readmission as well as lower mortality risk.(2) [B in Context Link 1]

[C] **National guidelines:** The Centers for Medicare and Medicaid Services (CMS) indicates that a face-to-face encounter must occur no more than 90 days prior to the home health start-of-care date or within 30 days of the start of the home healthcare as related to the primary reason the patient requires home health services. Supporting documentation requirements include the allowed provider type, the date of encounter, the need for skilled services, the patient's homebound status, and that the encounter was related to the primary reason the patient requires home health services.(4) [C in Context Link 1]

[D] **National guidelines:** The Centers for Medicare and Medicaid Services indicates that a Physical therapy (PT) referral, Occupational therapy (OT) referral, or Speech-language pathology (SLP) referral may occur once the patient is admitted to home care services by skilled nursing. If no skilled nursing services are on the home health plan of care, a PT or SLP may establish eligibility. However, an OT may establish eligibility under other programs (eg, Medicaid).(20) [D in Context Link 1]

[E] **National guidelines:** According to the Centers for Medicare and Medicaid Services (CMS), a maintenance therapy program helps to maintain the patient's current condition or to prevent or slow further deterioration. CMS specifies that skilled therapy services are necessary for the performance of a safe and effective maintenance program only when (a) the particular patient's special medical complications require the skills of a qualified therapist to perform a therapy service that would otherwise be considered nonskilled, or (b) the needed therapy procedures are of such complexity that the skills of a qualified therapist are required to perform the procedure.(4) [E in Context Link 1]

[F] **Evidence:** A systematic review of 15 studies including 1255 older adults found that patients receiving post-acute multidisciplinary rehabilitation had improved mobility 3 months after hospital discharge after an acute illness.(21) [F in Context Link 1]

[G] **Evidence:** A retrospective analysis of 3176 Medicare beneficiaries recovering from an ICU stay and discharged directly home with home healthcare found that factors reflecting greater rehabilitation needs were advanced age, greater post-hospital disability, and higher baseline dyspnea.(22) [G in Context Link 1]

[H] **Evidence:** A claims-based study of 1.4 million Medicare beneficiaries discharged from an acute care hospital to a post-acute care setting, including an inpatient rehabilitation facility (IRF), a skilled nursing facility (SNF), or home care (HC), found that more hours of occupational therapy and physical therapy services were associated with greater functional improvements, and fewer hours of occupational therapy and physical therapy services were associated with a higher risk for 30-day hospital readmission across all post-acute care settings.(23) [H in Context Link 1]

[I] **National guidelines:** The Centers for Medicare and Medicaid Services uses a data collection system to establish best practices, understand costs, and identify cost-effective systems and procedures. The current data collection system for eligible patients is the Outcome and Assessment Information Set (OASIS). OASIS is an assessment tool used to identify patient problems, strengths, and preferences. The OASIS used to create an individualized care plan can be found by going to www.cms.gov and searching OASIS Data Sets.(20) [I in Context Link 1]

[J] **Evidence:** A systematic review of 28 studies based on older cohorts of home healthcare patients found that skin ulcers, psychiatric conditions, dyspnea/COPD, cardiovascular conditions, diabetes, functional deficits, more comorbidities, and higher medication usage were associated with higher risk of hospital readmissions.(28) [J in Context Link 1]

[K] **Evidence:** A retrospective review of 19,179 patients discharged home after an elective lumbar fusion (eg, not due to malignancy or fracture) found that factors associated with postoperative adverse events and readmissions include presence of bleeding disorder, chronic steroid use, hypertension, insulin-dependent diabetes, higher body mass index, female sex, undergoing a complex surgery (eg, 2-level fusion, longer operative time), or having a pre-discharge complication.(29) [K in Context Link 1]

[L] **Evidence:** A retrospective review of 269,976 patients who underwent a lumbar fusion found that sociodemographic factors affecting 30-day and 90-day readmissions include having public insurance, black race, and undergoing procedure at low-volume hospitals.(30) [L in Context Link 1]

[M] Infection prevention and control measures are an important part of infectious disease management that should be maintained by all clinicians as well as an important component of the patient's self-management education. Hand hygiene, usage of personal protective equipment (PPE), and managing environment and clinical equipment are some cross-infection prevention techniques that should be maintained with every visit per organization protocol.(33) [M in Context Link 1]

[N] Naloxone rescue training and education about overdoses should be offered to the patient and family/caregiver as indicated.(43) [N in Context Link 1]

[O] Pain assessment should include how pain affects patient's physical and psychosocial function and progress toward treatment goals.(44) [O in Context Link 1]

[P] The patient's agreed-upon written treatment plan should include measurable and realistic functional or quality-of-life goals, pain-management education, treatment options, and safe use of opioid and nonopioid medications as prescribed.(43)(44) [P in Context Link 1]

[Q] **National guidelines:** The Centers for Disease Control and Prevention (CDC) recommends nonpharmacologic and nonopioid therapy before opioids for management of chronic pain (not due to malignancy). Considerations for opioids should include pain-management goal setting and determining if benefits outweigh risks. When opioids are prescribed, they should be provided in conjunction with nonpharmacologic pain-management interventions and planned monitoring for misuse or use escalation, including naloxone education.(43) [Q in Context Link 1]

Definitions

Custodial care

- Custodial care is personal unskilled care to support the patient's care of activities of daily living (ADL), such as bathing, dressing, and eating.(1)

References

1. Mauk KL. Post-acute care. In: Larsen PD, editor. Lubkin's Chronic Illness: Impact and Intervention. 11th ed. Jones & Bartlett Learning; 2022:477-520.

Home Health Aide (HHA) referral

- Home Health Aide (HHA) referral may be considered when **ALL** of the following exist(1)(2):
 - HHA supports skilled treatment plan (ie, facilitates treatment, prevents deterioration).
 - Nonskilled service needed for hands-on personal care (eg, bathing or dressing assistance), or other service to maintain patient's health (eg, repetitive practice exercises) or facilitate treatment (eg, simple dressing changes not requiring services of licensed nurse)
 - Significant functional limitations and:
 - Patient cannot perform service themselves.
 - There is no able and willing caregiver available to support patient care needs, or there is an able and willing caregiver available, but patient will not allow services to be performed by that caregiver.
 - Absence of functional plateau

References

1. Centers for Medicare and Medicaid Services. "Dependent services requirements." 42 CFR Pt. 409.45 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.
2. Centers for Medicare and Medicaid Services. "Condition of participation: Home health aide services." 42 CFR Pt. 484.80 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Homebound

- Patients are homebound if they have functional impairments that make leaving home a considerable taxing effort or are advised by their provider not to leave home. Homebound status may not be required if there are circumstances important for home monitoring of the patient's condition (eg, patient may need clinical evaluation of home regimen of chronic disease). Pediatric patients may have different considerations for homebound status, including infection prevention, dyspnea during activity, or home confinement for medical issues.[A](1)(2)(3)

References

1. Home Health Services. Chapter 7 Rev 11447 In: Medicare Benefit Policy Manual [Internet-Only Manual] [Internet] Centers for Medicare and Medicaid Services. 2022 Jun Accessed at: <https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Internet-Only-Manuals-IOMs-Items/CMS012673>. [accessed 2023 Sep 21]
2. Levels of care and care settings. In: Gillingham DC, editor. Foundations of Case Management. Blue Bayou Press; 2021:131-136.
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Footnotes

- A. **National guidelines:** The Centers for Medicare and Medicaid Services indicates that a considerable taxing effort generally includes the use of supportive devices such as crutches, canes, wheelchairs, and walkers; the use of special transportation; or the assistance of another person to leave home. A patient whose absences are infrequent, for short duration (eg, religious services, graduations), or necessary for healthcare (eg, dialysis, outpatient chemotherapy, medical day care) may still be considered homebound.(1)

Intermittent home care

- Intermittent home care is defined as part-time services that are provided in the home.^{[A](1)(2)(3)}

References

- Home Health Services. Chapter 7 Rev 11447 In: Medicare Benefit Policy Manual [Internet-Only Manual] [Internet] Centers for Medicare and Medicaid Services. 2022 Jun Accessed at: <https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Internet-Only-Manuals-IOMs-Items/CMS012673>. [accessed 2023 Sep 21]
- Deutsch A. Health policy. In: Larsen PD, editor. Lubkin's Chronic Illness: Impact and Intervention. 11th ed. Jones & Bartlett Learning; 2022:569-584.
- Community-based services. In: Chies S, editor. Pratt's Long-Term Care. 5th ed. Jones & Bartlett Learning; 2022:183-218.

Footnotes

- A. **National guidelines:** The Centers for Medicare and Medicaid Services defines intermittent care as services that must be less than 8 hours each day and 28 or fewer hours each week (or, for special cases, up to 35 hours each week).⁽¹⁾

Occupational therapy (OT) progression

- OT progression includes **ALL** of the following⁽¹⁾⁽²⁾:
 - Ongoing evaluation of treatment plan with short-term and long-term goals
 - Ongoing evaluation of rehabilitation potential toward achieving goals
 - Patient demonstrates resolution of barriers to transition of care and **1 or more** of the following:
 - Measured improvements in performance of short-term goals in reasonable and predictable time frame based on treatment plan^{[A](4)(5)}
 - Minimal progress due to unexpected clinical event, but progress expected to resume toward goals as condition improves

References

- Bennett K. Setting rehabilitation goals. In: O'Hanlon S, Smith M, editors. Comprehensive Guide to Rehabilitation of the Older Patient. 4th ed. Elsevier; 2021:49-52.
- Pryor J, O'Reilly K, Bonser M, Garret G, McKenchnie D. Rehabilitation for the individual and family. In: Chang E, Johnson A, editors. Living With Chronic Illness and Disability. 4th ed. Chatswood NSW 2067: Elsevier; 2022:161-182.
- Yang W, Houtrow A, Cull DS, Annaswamy TM. Quality and outcome measures for medical rehabilitation. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:100-114.e2.
- Kee YYK. Measuring progress with rehabilitation. In: O'Hanlon S, Smith M, editors. Comprehensive Guide to Rehabilitation of the Older Patient. 4th ed. Elsevier; 2021:111-115.
- Barker K, Eickmeyer S. Therapeutic exercise. Medical Clinics of North America 2020;104(2):189-198. DOI: 10.1016/j.mcna.2019.10.003.

Footnotes

- A. Improvements may be measured by accuracy (percentage or number of correct trials), increased number of repetitions, decreased assistance level (maximum, moderate, or minimal assistance), decreased pain level, increased ROM/strength, increased balance scores, decreased level of cues or prompts required (in conjunction with level of assistance), or improvement in standardized functional outcome measures (eg, FIM®, WeeFIM®, Tinetti, Berg Balance Scale).⁽³⁾⁽⁴⁾ Generally, it is expected that measured improvements should be demonstrable in targeted areas over a 1-week to 2-week period of skilled therapy. The speed of recovery is variable and may be linked to intensity of treatment, patient condition, age, comorbidities, and other factors.

Occupational therapy (OT) referral

- Occupational therapy (OT) referral may be considered when **ALL** of the following exist⁽¹⁾:
 - Skills of therapist required to assess functional needs and limitations, rehabilitation potential, and to develop or implement therapy plan of care⁽²⁾
 - Patient able to make measurable improvement in condition within reasonable time frame⁽³⁾
 - Need for evaluation, training, or education related to functional decline for **1 or more** of the following⁽⁴⁾⁽⁵⁾:
 - Decreased independence or safety in performance of ADL or IADL. See Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment for more information.
 - Mental function changes
 - Neuromusculoskeletal and movement-related functions
 - Sensory function and pain
 - Cardiovascular, integumentary, and respiratory system function

References

1. American Occupational Therapy Association, Inc. Occupational Therapy Practice Framework: Domain & Process. 4th ed. AOTA Press, 2020.
2. Centers for Medicare and Medicaid Services. "Examples of skilled nursing and rehabilitation services." 42 CFR Pt. 409.33 Washington, DC 2023 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.
3. Yang W, Houtrow A, Cull DS, Annaswamy TM. Quality and outcome measures for medical rehabilitation. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:100-114.e2.
4. Home care: a unique specialty. In: Marrelli TM, St. Pierre M, editors. Handbook of Home Health Standards. 6th ed. St. Louis, MO: Elsevier Mosby; 2018:1-100.
5. Barker KDD, Johnson MM. The psychiatric history and physical examination. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:1-41.e2.

Patient/caregiver able to monitor and self-manage condition, or unwilling or unable to be trained after sufficient efforts made

- **National Guideline:** The Centers for Medicare and Medicaid Services indicates that continued teaching and training is not reasonable if the patient and/or caregiver cannot or will not be trained in an appropriate time frame. In these situations, considerations for an alternative level of care and referrals to community resources are appropriate to meet patient needs.(1)

References

1. Centers for Medicare and Medicaid Services. "Beneficiary qualifications for coverage of services." 42 CFR Pt. 409.42 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Physical therapy (PT) progression

- PT progression includes **ALL** of the following(1)(2)(3):
 - Ongoing evaluation of treatment plan with short-term and long-term goals
 - Ongoing evaluation of rehabilitation potential toward achieving goals
 - Patient demonstrates resolving of barriers to transition of care and **1 or more** of the following:
 - Measured improvements in performance of short-term goals in reasonable and predictable time frame based on treatment plan(A)(4)(5)
 - Minimal progress due to unexpected clinical event, but progress expected to resume toward goals as condition improves

References

1. Malone DJ. Introduction to physical therapist management of the acute care patient. In: Malone DJ, Bishop KL, editors. Acute Care Physical Therapy. 2nd ed. Slack, Inc.; 2020:1-50.
2. Bartels MN, Prince DZ. Acute medical conditions: cardiopulmonary disease, medical frailty, and renal failure. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:511-534.e5.
3. Centers for Medicare and Medicaid Services. "Skilled services requirements." 42 CFR Pt. 409.44 Washington, DC 2023 Oct [accessed 2023 Oct 04] Accessed at: <https://www.ecfr.gov/>.
4. Yang W, Houtrow A, Cull DS, Annaswamy TM. Quality and outcome measures for medical rehabilitation. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:100-114.e2.
5. Barker K, Eickmeyer S. Therapeutic exercise. Medical Clinics of North America 2020;104(2):189-198. DOI: 10.1016/j.mcna.2019.10.003.

Footnotes

- A. Improvements may be measured by accuracy (percentage or number of correct trials), increased number of repetitions, decreased assistance level (maximum, moderate, or minimal assistance), decreased pain level, increased ROM/strength, increased balance scores, decreased level of cues or prompts required (in conjunction with level of assistance), or improvement in standardized functional outcome measures (eg, FIM®, WeeFIM®, Tinetti, Berg Balance Scale).(1)(4) Generally, it is expected that measured improvements should be demonstrable in targeted areas over a 1-week to 2-week period of skilled therapy. The speed of recovery is variable and may be linked to intensity of treatment, patient condition, age, comorbidities, and other factors.

Physical therapy (PT) referral

- Physical therapy (PT) referral may be considered when **ALL** of the following exist(1):
 - Skills of therapist required to assess functional needs and limitations, rehabilitation potential, and to develop or implement therapy plan of care(2)
 - Patient able to make measurable improvement in condition within reasonable and predictable time frame(3)
 - Need for evaluation, training, or education related to functional decline for **1 or more** of the following(4):

- Aerobic capacity and endurance
- Ambulation or mobility
- Balance or coordination
- Contracture management (education, splint, positioning)
- Decreased independence or safety in performance of ADL or IADL. See Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment for more information.
- Developmental progression (ie, pediatric developmental delay)(5)
- Durable medical equipment (DME) or orthotic device management
- Environmental and home barriers
- Ergonomics and body mechanics
- Functional mobility (eg, bed and transfer training)
- Level of care planning (eg, home exercise program vs outpatient therapy)
- Motor function (motor control and motor learning)
- Muscle performance (range of motion or strength)
- Pain or edema management
- Self-care management
- Wound management

References

1. Malone DJ. Introduction to physical therapist management of the acute care patient. In: Malone DJ, Bishop KL, editors. Acute Care Physical Therapy. 2nd ed. Slack, Inc.; 2020:1-50.
2. Centers for Medicare and Medicaid Services. "Examples of skilled nursing and rehabilitation services." 42 CFR Pt. 409.33 Washington, DC 2023 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.
3. Centers for Medicare and Medicaid Services. "Skilled services requirements." 42 CFR Pt. 409.44 Washington, DC 2023 Oct [accessed 2023 Oct 04] Accessed at: <https://www.ecfr.gov/>.
4. Keenan MAE, McMahon PJ. Rehabilitation. In: McMahon PJ, Skinner HB, editors. Current Diagnosis & Treatment in Orthopedics. 6th ed. McGraw-Hill Medical; 2021:528-580.
5. Shapiro BK, O'Neill ME. Developmental delay and intellectual disability. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:283-294.e1.

Rehabilitation program completed for safe transfer to lower level of care or patient no longer demonstrating significant functional gains

- **National guideline:** Rehabilitation services are not considered skilled for the supervision of taught exercises, repetitious exercise administration (unless there is a loss of function), or are provided for assistance only.(1)

References

1. Centers for Medicare and Medicaid Services. "Examples of skilled nursing and rehabilitation services." 42 CFR Pt. 409.33 Washington, DC 2023 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Safe at home

- Home safety may be determined by a completed Home Safety Assessment and the patient being in no imminent harm. Harm includes high risk for deterioration or injury and may include risk of harm to self or others.(1)(2)

References

1. Laplante N. Home care safety. In: Perry AG, Potter PA, Ostendorf WR, editors. Nursing Interventions and Clinical Skills. 7th ed. Elsevier; 2020:842-862.
2. Ekroos RA, Traveller L. Violence and abuse. In: Rector C, Stanley MJ, editors. Community and Public Health Nursing Promoting the Public's Health. 10th ed. Wolters Kluwer; 2022:506-536.

Skilled care

- Skilled care is that which must be performed or supervised by professional or technical personnel, be reasonable and necessary for patient's treatment, and exceed scope of Custodial care. Skilled care may be direct (eg, in-person assessment and administration of treatments) or indirect (eg, coordinating care between providers, supervising care aides).[A](1)(2)(3)(4)

References

1. Centers for Medicare and Medicaid Services. "Criteria for skilled services and the need for skilled service." 42 CFR Pt. 409.32 Washington, DC 2023 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.
2. Harding MM. Professional nursing. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:1-18.

3. Medical-surgical nursing. In: Hinkle JL, Cheever KH, Overbaugh KJ, editors. Brunner & Suddarth's Textbook of Medical-Surgical Nursing. 15th ed. Wolters Kluwer; 2022:33-55.
4. Centers for Medicare and Medicaid Services. Medicare Program Integrity Manual. Chapter 6 - Medical Review of Inpatient Hospital Claims for Part A Payment Rev. 10365 [Internet] Centers for Medicare and Medicaid Services. 2020 Oct 02 Accessed at: <https://www.cms.gov/regulations-and-guidance/regulations-and-guidance>. [accessed 2023 Aug 31]

Footnotes

- A. **National guidelines:** The Centers for Medicare and Medicaid Services defines skilled care as a service that is "so inherently complex that it can only be safely and effectively performed by, or under the supervision of, professional or technical personnel." (1)

Social worker referral

- Social worker (SW) referral may be considered when **1 or more** of the following exist^{(A)(3)(4)}:
 - Social or emotional problems that impede medical treatment or recovery⁽⁵⁾
 - Abuse or neglect suspected
 - Advance care planning (eg, end-of-life care, advance directives)
 - Financial or legal assistance, transportation, or housing referrals needed
 - Decision-making facilitation needed within family or among caregivers

References

1. Gandy J. Legal and ethical basis for practice. In: Fosbre CD, Varcacolis EM, Chiappetta L, editors. Varcacolis' Essentials of Psychiatric-Mental Health Nursing. 5th ed. Elsevier; 2023:60-73.
2. Barker KDD, Johnson MM. The psychiatric history and physical examination. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:1-41.e2.
3. de Wilde C. Social work. In: O'Hanlon S, Smith M, editors. Comprehensive Guide to Rehabilitation of the Older Patient. 4th ed. Elsevier; 2021:91-94.
4. Harding MM. Professional nursing. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:1-18.
5. Centers for Medicare and Medicaid Services. "Dependent services requirements." 42 CFR Pt. 409.45 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Footnotes

- A. Social worker (SW) subspecialties include medical, mental health, and substance use. The type of SW referral should include consideration of subspecialty, diagnosis of the patient, and the purpose for referral. (1)(2)

Speech-language pathology (SLP) referral

- Speech-language pathology (SLP) therapy referral may be considered when **ALL** of the following exist:
 - Skills of therapist required to assess functional needs and limitations, rehabilitation potential, and to develop or implement therapy plan of care⁽¹⁾
 - Patient able to make measurable improvement in condition within reasonable time frame⁽²⁾
 - Patient needs evaluation, training, or education related to functional decline for **1 or more** of the following⁽³⁾⁽⁴⁾:
 - Speech
 - Language
 - Voice
 - Swallowing or chewing
 - Hearing as it relates to communication⁽⁵⁾⁽⁶⁾
 - Cognitive-communication
 - Developmental progression (ie, pediatric developmental delay)⁽⁷⁾
 - Augmentative/alternative communication system development, modification, training, and use⁽²⁾

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7. Shapiro BK, O'Neill ME. Developmental delay and intellectual disability. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:283-294.e1.

Transition of care planning completed

- Transition of care planning completed, including **ALL** of the following(1)(2):
 - Transition plan communicated to all members of patient's healthcare team
 - Summary of care, discharge list of medications, and transition plan communicated to primary care provider
 - Medication reconciliation complete
 - Education on condition management and complications to report provided to patient or caregiver[A]
 - Follow-up appointments scheduled
 - Referrals to continue medical and rehabilitation goals (eg, outpatient) arranged, if needed
 - Services (eg, equipment, environment modifications, transportation) arranged, if needed
 - Psychosocial issues addressed, or plan for management, if needed

References

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Footnotes

- A. Health education materials should be given in patient's and caregiver's native language using trained language interpreters whenever possible.(3)

Transition plans understood

- Transition plans and education understood, including(1)(2):
 - Patient and caregiver show ability and understanding to manage condition and treatments.
 - Patient and caregiver know signs and symptoms of complications, whom to contact, and how and when to seek medical intervention.

References

1. Morris BC. Patient and caregiver teaching. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:49-62.
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Codes

ICD-10 Diagnosis: A18.01, C41.2, C79.40, C79.49, C79.51, D16.6, D32.1, D33.4, D42.1, D43.4, D48.0, D49.2, G06.1, G80.0, G80.1, G80.2, G83.4, K59.2, M08.1, M43.05, M43.06, M43.07, M43.09, M43.15, M43.16, M43.17, M43.19, M45.5, M45.6, M45.7, M46.25, M46.26, M46.27, M46.35, M46.36, M46.37, M46.45, M46.46, M46.47, M47.15, M47.16, M47.25, M47.26, M47.27, M47.815, M47.816, M47.817, M47.895, M47.896, M47.897, M48.05, M48.061, M48.062, M48.07, M48.8X5, M48.8X6, M48.8X7, M51.05, M51.06, M51.15, M51.16, M51.17, M51.25, M51.26, M51.27, M51.35, M51.36, M51.37, M51.85, M51.86, M51.87, M51.A0, M51.A1, M51.A2, M51.A3, M51.A4, M51.A5, M54.15, M54.16, M54.17, M96.1, M99.13, M99.23, M99.33, M99.43, M99.53, M99.63, M99.73, Q76.2, S33.0XXA, S33.0XXD, S33.0XXS, S33.100A, S33.100D, S33.100S, S33.101A, S33.101D, S33.101S, S33.110A, S33.110D, S33.110S, S33.111A, S33.111D, S33.111S, S33.120A, S33.120D, S33.120S, S33.121A, S33.121D, S33.121S, S33.130A, S33.130D, S33.130S, S33.131A, S33.131D, S33.131S, S33.140A, S33.140D, S33.140S, S33.141A, S33.141D, S33.141S, S34.101A, S34.101D, S34.101S, S34.102A, S34.102D, S34.102S, S34.103A, S34.103D, S34.103S, S34.104A, S34.104D, S34.104S, S34.105A, S34.105D, S34.105S, S34.109A, S34.109D, S34.109S, S34.111A, S34.111D, S34.111S, S34.112A, S34.112D, S34.112S, S34.113A, S34.113D, S34.113S, S34.114A, S34.114D, S34.114S, S34.115A, S34.115D, S34.115S, S34.119A, S34.119D, S34.119S, S34.121A, S34.121D, S34.121S, S34.122A, S34.122D, S34.122S, S34.123A, S34.123D, S34.123S, S34.124A, S34.124D, S34.124S, S34.125A, S34.125D, S34.125S, S34.129A, S34.129D, S34.129S, S34.21XA, S34.21XD, S34.21XS, S34.3XXA, S34.3XXD, S34.3XXS, Z45.1, Z45.2, Z48.01, Z48.89 [Hide]

ICD-10 Procedure: 00NY0ZZ, 00NY3ZZ, 00NY4ZZ, 01NB0ZZ, 01NB3ZZ, 01NB4ZZ, 01NN0ZZ, 01NN3ZZ, 01NN4ZZ, 0RBB0ZZ, 0RBB3ZX, 0RBB3ZZ, 0RBB4ZX, 0RBB4ZZ, 0RGA070, 0RGA071, 0RGA07J, 0RGA0A0, 0RGA0AJ, 0RGA0J0, 0RGA0J1, 0RGA0JJ, 0RGA0K0, 0RGA0K1, 0RGA0KJ, 0RGA370, 0RGA371, 0RGA37J, 0RGA3A0, 0RGA3AJ, 0RGA3J0, 0RGA3J1, 0RGA3JJ, 0RGA3K0, 0RGA3K1, 0RGA3KJ, 0RTB0ZZ, 0SB20ZX, 0SB20ZZ, 0SB23ZX, 0SB23ZZ, 0SB24ZX, 0SB24ZZ, 0SB40ZX, 0SB40ZZ, 0SB43ZX, 0SB43ZZ, 0SB44ZX, 0SB44ZZ, 0SG0070, 0SG0071, 0SG007J, 0SG00A0, 0SG00AJ, 0SG00J0, 0SG00J1, 0SG00JJ, 0SG00K0, 0SG00K1, 0SG00KJ, 0SG0370, 0SG0371, 0SG037J, 0SG03A0, 0SG03AJ, 0SG03J0, 0SG03J1, 0SG03JJ, 0SG03K0, 0SG03K1, 0SG03KJ, 0SG1070, 0SG1071, 0SG107J, 0SG10A0, 0SG10AJ, 0SG10J0, 0SG10J1, 0SG10JJ, 0SG10K0, 0SG10K1, 0SG10KJ, 0SG1370, 0SG1371, 0SG137J, 0SG13A0, 0SG13AJ, 0SG13J0, 0SG13J1, 0SG13JJ, 0SG13K0, 0SG13K1, 0SG13KJ, 0SG3070, 0SG3071, 0SG307J, 0SG30A0, 0SG30AJ, 0SG30J0, 0SG30J1, 0SG30JJ, 0SG30K0, 0SG30K1, 0SG30KJ, 0SG3370, 0SG3371, 0SG337J, 0SG33A0, 0SG33AJ, 0SG33J0, 0SG33J1, 0SG33JJ, 0SG33K0, 0SG33K1, 0SG33KJ, 0ST20ZZ, 0ST40ZZ, XRGA0R7, XRGA3R7, XRGB0R7, XRGB3R7, XRG00R7, XRG03R7, XRGD0R7, XRGD3R7 [Hide]

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